SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Vemurafenib alone — advanced melanoma

Advanced melanoma · NCCP 00102 · version 6 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
To control advanced BRAF V600-mutated melanoma with a daily oral targeted medicine; benefit and tolerability are reviewed for the individual.

Schedule
Vemurafenib twice daily, about 12 hours apart, every day. A 28-day review cycle does not mean a planned tablet-free week.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Vemurafenib

BRAF TARGETED TREATMENT

- ☐ Photosensitivity, rash and new skin cancers
- ☐ Joint pain and tiredness
- ☐ Nausea, diarrhoea and dehydration
- ☐ Heart rhythm changes
- ☐ Liver, kidney and eye problems

Discuss the exact diagnosis and treatment intent, expected benefit and uncertainty, alternatives, regimen-specific risks, tests, fertility, practical burdens and your preferences. This guide supports the consent conversation and does not replace it. This is vemurafenib monotherapy. It is not the cobimetinib combination and has no cobimetinib seven-day break. The team must plan any radiotherapy because BRAF treatment can increase radiation-related reactions.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00102 v6.

SACTCheck v0.84.42
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